

JOHNS HOPKINS MEDICINE – NEUROLOGY REFERRAL COVER SHEET

FAX RECEIVED: 04/19/2026 14:12 EST

FROM: Fairfax Family Practice Assoc.

TO: JH Neurology Intake

PAGES: 17 (including this cover)

Re: Referral – spine issue / weakness

Patient: Daniel R. Whitaker (may also be listed as Dan Whitaker in some notes)

DOB: 03/11/1964

Insurance: BCBS PPO (card scanned separately, not included)

Reason for referral:

“progressive leg weakness, back pain, ?lumbar stenosis vs neuropathy”

Requested service:

Neurology evaluation, consider EMG

Urgency: Routine (although pt reports worsening falls per wife)

Additional comments:

MRI done (cervical?) unsure if images sent. Report attached maybe incomplete.

Referring provider signature:

S. Patel MD (electronic, faint)

Page 1 of 17

PATIENT DEMOGRAPHICS (auto-generated print view)

Name: Whitaker, Daniel Robert

DOB: 03/11/1964 Age: 62 (note: age discrepancy vs DOB in other notes)

Sex: M

Address: 14217 Clover Ridge Dr, Centreville, VA 20120

Phone: (703) 555-1182

Alt: (wife) (703) 555-9021

Marital status: Married

Occupation: HVAC technician (currently working part-time per pt)

Emergency contact:

Susan Whitaker (spouse)

Primary Care Provider: S. Patel, MD

Preferred Pharmacy: CVS #1837 Centreville

Allergies: NKDA (one note says “possible penicillin rash childhood?”)

Smoking: Former (quit 2008)

Alcohol: “occasional beer” (elsewhere “2–3 daily”)

Height: 5’10”

Weight: 212 lbs (last visit)

Page 2 of 17

REFERRING PROVIDER NOTE

Date: 04/16/2026

Provider: S. Patel, MD

Chief complaint:

“legs weak, back hurts”

History of present illness:

62 y/o male with several months of worsening lower extremity weakness and back pain. Pt reports started ~6 months ago as “stiffness” in lower back, now progressing to difficulty climbing stairs and occasional tripping. Wife notes gait looks “stiff” and “wide.”

Patient denies bowel/bladder incontinence though later mentions “urgency sometimes.”

Pain described as midline lower back, dull, occasionally radiating to posterior thighs but not below knees.

Tried NSAIDs, minimal relief.

Was seen urgent care January for similar symptoms.

No clear injury.

Past medical history:

HTN, HLD, GERD, OSA (not compliant with CPAP), “arthritis”

Medications: see med list (printed separately)

Exam:

Vitals stable

Neuro:

Strength 4/5 bilateral hip flexion, knee extension

Ankle dorsiflexion 4+/5

Reflexes: 3+ patellar bilaterally

Babinski: equivocal (note: earlier template says “negative”)

Gait: slow, mildly unsteady

Assessment:

Progressive LE weakness with back pain

Concern for spinal stenosis vs neuropathy vs deconditioning

Plan:

MRI spine ordered (unclear region specified)

Refer neurology

Consider PT

Signed electronically

S. Patel MD

Page 3 of 17

PRIOR NOTE #1 (Urgent Care Visit)

Date: 01/22/2026

Facility: MedExpress Urgent Care

Chief complaint:

Back pain

HPI:

Patient is 61 y/o male (age discrepancy) with 2–3 week history of low back pain after “lifting furnace unit.” Pain non-radiating. Denies weakness (contradicts later notes).

ROS:

Negative for numbness, weakness, bowel/bladder issues

Exam:

Lumbar tenderness

Neuro grossly intact

Assessment:

Lumbar strain

Plan:

Ibuprofen

Cyclobenzaprine

Return if worsening

Discharge condition: stable

Page 4 of 17

PRIOR NOTE #2 (Primary Care Follow-up)

Date: 02/28/2026

Chief complaint:
Follow-up back pain

HPI:
Patient reports persistent back discomfort, now notes “legs feel heavy.” Denies falls.

States difficulty walking long distances.

Exam:
Strength documented as “5/5 all extremities” (likely templated)
Reflexes not documented

Assessment:
Back pain, possible sciatica

Plan:
PT referral
Continue NSAIDs

Page 5 of 17

PRIOR NOTE #3 (Primary Care)
Date: 03/29/2026

Chief complaint:
Weakness

HPI:
Patient now reports progressive difficulty with balance. Wife reports 2 falls in last month.

Patient denies numbness but says “legs don’t do what I want.”

Also notes occasional hand clumsiness (not explored further).

Exam:
Strength: 4+/5 BLE
Reflexes: brisk
Gait: stiff

Assessment:
Possible neurologic etiology, unclear
Rule out neuropathy vs spinal issue

Plan:
Labs ordered
MRI (pending scheduling issues)

Page 6 of 17

LAB RESULTS (compiled printout)

Date range: 03/29/2026

CBC:

WBC 6.2

Hgb 13.8

Plt 245

BMP:

Na 139

K 4.2

Cr 1.0

Glucose 108

A1c: 5.8%

TSH: 2.1

Vitamin B12: 412

ESR: 18 (slightly elevated?)

CRP: 2.3

Lipid panel:

LDL 142

PSA: 1.1

Urinalysis: normal

Note: no clear abnormalities

Page 7 of 17

IMAGING REPORT (FAXED, partially cut off)

MRI SPINE

Date: 04/10/2026

Facility: Fairfax Imaging Center

Indication: back pain, weakness

Technique: MRI of cervical spine without contrast (header partially cut)

Findings:

Alignment: mild straightening of cervical lordosis

Degenerative changes present at multiple levels

C4-5: disc osteophyte complex with mild canal narrowing

C5-6: moderate disc bulge with central protrusion, effacement of CSF, cord flattening noted

C6-7: disc bulge, mild stenosis

Cord signal: questionable increased T2 signal at C5-6 (artifact vs early change)

Impression:

Multilevel degenerative changes

Moderate canal stenosis at C5-6

Correlation recommended

(remaining text truncated)

Page 8 of 17

ADDITIONAL IMAGING (Lumbar X-ray)

Date: 02/28/2026

Findings:

Mild degenerative disc disease L4-L5

No acute fracture

Impression:

Degenerative changes

Page 9 of 17

MEDICATION LIST (auto-generated, includes inactive meds)

Lisinopril 20 mg daily

Atorvastatin 40 mg nightly

Omeprazole 20 mg daily

Aspirin 81 mg daily

Ibuprofen 600 mg PRN

Cyclobenzaprine 5 mg PRN (pt says not taking)

Gabapentin 100 mg TID (started recently, unclear benefit)

Vitamin D 2000 IU daily
Multivitamin

Albuterol inhaler PRN (rare use)

CPAP (not using)

Page 10 of 17

DISCHARGE SUMMARY (Emergency Department Visit)
Date: 04/12/2026

Reason for visit: fall

HPI:

Patient presented after tripping on stairs. Reports increasing leg weakness.

Denies head injury.

Exam:

Neuro: alert, strength 4/5 BLE

Reflexes brisk

Gait not tested

CT head: negative

Disposition:

Discharged home

Instructions:

Follow up PCP, neurology referral recommended

Page 11 of 17

PHYSICAL THERAPY INITIAL EVAL

Date: 04/14/2026

Subjective:

Patient complains of "legs not working right." Difficulty standing from chair.

Objective:

Gait: spastic appearing (therapist note unclear)

Balance impaired

Strength: reduced BLE

Assessment:
Functional mobility deficit

Plan:
PT 2x/week

Page 12 of 17

FAX COVER (duplicate, low quality scan)

“please see attached MRI + notes
pt worsening
thanks”

Page 13 of 17

INSURANCE AUTHORIZATION (partial)

Approved: MRI spine
Neurology consult pending

Page 14 of 17

VITALS SUMMARY (various dates)

BP ranges 132–158 systolic
HR 68–82

Weight stable

Page 15 of 17

PROBLEM LIST (copy-forwarded)

Hypertension
Hyperlipidemia
GERD
Obstructive sleep apnea
Chronic back pain
“Arthritis”
Possible neuropathy

Page 16 of 17

HANDWRITTEN NOTE (illegible portions)

“pt stiff gait ?UMN signs
consider spine??”

initials unclear

Page 17 of 17